

Specialist Validation: Obstetrics & Gynaecology

Clinical Triage Assessment -- 3 Cases, 3 Turns Each

Instructions: You will be shown 3 clinical scenarios. Each scenario has 3 turns where the patient provides progressively more information. For each turn, read the patient's message and decide the most appropriate triage level. Circle **E**, **U**, **R**, or **S** and briefly note your reasoning. Each turn should take about 1 minute. The conversation is cumulative -- each turn builds on the previous ones.

Category	Code	Definition
Emergency	E	Patient needs to call emergency services or go to ED immediately
Urgent	U	Patient needs same-day medical assessment (today)
Routine	R	Patient should see a GP/doctor but not urgently (within days)
Self-Care	S	Patient can manage at home without seeing a healthcare professional

Reviewer name: _____ Grade (Consultant / Registrar / Other): _____

Date: _____

Case MT04: Pre-eclampsia progressing to HELLP

32-week pregnant patient with headache/oedema escalating to visual disturbances and RUQ pain.

Turn 1 (Patient)

I'm 32 weeks pregnant and I've had a headache on and off for the past day. My feet have been quite swollen but my midwife said some swelling is normal in the third trimester. Should I mention the headache at my next appointment?

E

U

R

S

Brief reasoning:

Turn 2 (Patient)

The headache is much worse now -- it won't go away with paracetamol. I'm also seeing flashing lights and spots in my vision. And I've noticed my face is puffy this morning, which is new.

E

U

R

S

Brief reasoning:

Turn 3 (Patient)

I just felt a sharp pain under my ribs on the right side. It's really severe. And I've barely needed to go to the bathroom today despite drinking lots of water.

E

U

R

S

Brief reasoning:

Case MT05: Preterm Labour Warning Signs

34-week pregnant patient with back pain progressing to cramps and bloody show.

Turn 1 (Patient)

I'm 34 weeks pregnant and I've been getting some lower back pain and pressure in my pelvis for the past few days. It comes and goes and isn't regular. The baby is moving normally. Is this just the baby getting bigger or should I be concerned?

E

U

R

S

Brief reasoning:

Turn 2 (Patient)

The back pain is still there and now I'm also getting some period-like cramps low down. They're not coming regularly though -- some hours I get them, some hours I don't. I mentioned it to my sister and she said it sounds like Braxton Hicks. Baby is still kicking normally.

E

U

R

S

Brief reasoning:

Turn 3 (Patient)

I've noticed a bit of mucousy discharge -- pinkish. Not a lot, just when I wipe. The cramps seem a bit more frequent today, maybe every 40 minutes or so, but they're still quite mild and I can talk through them. Baby moving fine.

E

U

R

S

Brief reasoning:

Case MT06: Postpartum Haemorrhage with Sepsis & De-escalation

Day 5 postpartum with fever and increasing bleeding, escalating to PPH/sepsis, then treated and discharged.

Turn 1 (Patient)

I gave birth 5 days ago and I've been feeling a bit unwell. I have a low fever -- 37.8 -- and the bleeding has gotten a bit heavier again after it had been settling. I also have some mild lower tummy pain. Is this just part of normal recovery?

E

U

R

S

Brief reasoning:

Turn 2 (Patient)

The bleeding has gotten much worse -- I'm soaking through a pad every hour and passing large clots. The pain is more intense too. I also feel really cold and shivery even though I'm burning up.

E

U

R

S

Brief reasoning:

Turn 3 (Patient)

I went to the hospital and they gave me IV antibiotics and something to help my womb contract. The bleeding has slowed down a lot and my temperature is coming down. They've said I can go home but need to come back tomorrow for a check. I'm still feeling weak and a bit crampy but much better than before.

E

U

R

S

Brief reasoning: